

--SUMMARY--

Decision No. 24/19

18-Mar-2019

R.Hoare

- Consequences of injury (altered gait)
- Permanent impairment {NEL} (redetermination) (significant deterioration)

No Summary Available

16 Pages

References: Act Citation

- WSIA

Other Case Reference

- [w1919n]

Style of Cause:

Neutral Citation: 2019 ONWSIAT 683



WORKPLACE SAFETY AND INSURANCE APPEALS TRIBUNAL

DECISION NO. 24/19

BEFORE:

R.M.J. Hoare: Vice-Chair

HEARING:

January 8, 2019 at Toronto
Oral

DATE OF DECISION:

March 18, 2019

NEUTRAL CITATION:

2019 ONWSIAT 683

DECISION UNDER APPEAL:

WSIB Appeals Resolution Officer (ARO), A. Danos, dated
October 23, 2017

APPEARANCES:

For the worker:

P. Carvalho, Paralegal

For the employer:

T. McDonald, Paralegal

Interpreter:

N/A

REASONS

(i) Introduction

[1] The worker appeals the decision of the Appeals Resolution Officer (ARO), A. Danos, dated October 23, 2017. The ARO rendered a decision based upon the written record without an oral hearing.

[2] The ARO's decision concluded that the worker was not entitled to bilateral hand, wrist, thumb and right knee injuries as secondary conditions arising from a compensable left knee injury. The decision also concluded that the worker was not entitled to a redetermination of the 11% quantum of her Non-Economic Loss (NEL) award for her left knee based on a significant deterioration in her left knee condition subsequent to the date of the original NEL award.

[3] On August 23, 2006, the worker was granted a 66% NEL award of which the 11% NEL award for her left knee was a component.

(ii) Issues

[4] The issues to be determined in this case are as follows:

1. Whether the worker should have entitlement for the bilateral hands, wrist and thumbs and right knee as secondary conditions;
2. Whether the worker should have entitlement to a redetermination of her 11% NEL award for her left knee due to a significant deterioration of her left knee condition subsequent to the NEL award of August 23, 2006.

(iii) Synopsis of the appeal

[5] The now 54 year old worker was working as an educational assistant for the accident employer, a school board. On December 20, 2002, she was working with a six year old male student with behavioural problems. In the course of her interactions with the child, she attempted to subdue him as his violent behavior was escalating in intensity and she was concerned he may harm himself and other students. She was injured as a result of a physical altercation when the student assaulted her as she attempted to check his behavior. She strained various parts of her body. She also sustained a nasal fracture when the child head butted her.

[6] Entitlement was granted by the Workplace Safety and Insurance Board (WSIB) in relation to the December 20, 2002 workplace accident for cervical strain, nasal fracture, bilateral shoulder strain, bilateral forearm soft tissue injuries, aggravation of an existing migraine headache condition and Psychotraumatic disability for various psychological conditions (Post Traumatic Stress Disorder (PTSD) and major depressive disorder).

[7] On February 4, 2004, while attending a WSIB sponsored Psychological Trauma Program (PTP) to address her injuries from the workplace accident, the worker fell on a snowbank after leaving a taxi which had dropped her off at the hospital where the PTP was held.

[8] As a result of this February 4, 2004 accident, the WSIB granted her entitlement for left knee strain, lower back strain, left ankle strain and left small toe fracture as secondary conditions.

[9] The various claims at issue in this appeal arise in relation to, and are claimed to be related to, the compensable injury to the worker's left knee that occurred in the accident dated February 4, 2004.

(iv) Law and policy

[10] Since the worker was injured in 2002 and 2004, the *Workplace Safety and Insurance Act, 1997* (the WSIA) is applicable to this appeal. All statutory references in this decision are to the WSIA, as amended, unless otherwise stated.

[11] Tribunal jurisprudence applies the test of significant contribution to questions of causation. A significant contributing factor is one of considerable effect or importance. It need not be the sole contributing factor. See, for example, *Decision No. 280*.

[12] Pursuant to section 126 of the WSIA, the Board stated that the following policy packages, Revision #9, would apply to the subject matter of this appeal:

- #12 - Adjudicative Process
- #30 - Adjusting Benefits for Non-work-related Change in Circumstances
- #31 - Secondary Conditions
- #214 - NEL Redetermination
- #241 - Initial Entitlement
- #300 - Decision Making/Benefit of Doubt/Merits and Justice.

[13] I have considered these policies as necessary in deciding the issues in this appeal, in particular:

- *Operational Policy Manual* (OPM) Document No.15-05-01, "Secondary Conditions-Resulting From Work-Related Disability/Impairment;"
- OPM Document No.18-05-09, "NEL Redeterminations"

[14] Section 46 of the WSIA provides that if a worker's injury results in permanent impairment, the worker is entitled to compensation for non-economic loss.

[15] "Impairment" means a physical or functional abnormality or loss (including disfigurement) which results from an injury and any psychological damage arising from the abnormality or loss.

[16] "Permanent impairment" means impairment that continues to exist after the worker reaches maximum medical recovery.

[17] Legislation and Board policy provide that the degree of a worker's permanent impairment is determined in accordance with the prescribed rating schedule or criteria, any medical assessments, and having regard to the health information on file. The prescribed rating schedule for most impairments is the American Medical Association's *Guides to the Evaluation of Permanent Impairment*, 3rd edition (revised) (the AMA Guides). The Board has adopted specific rating schedules for impairment due to psychological disability, fibromyalgia, chronic pain and other conditions.

[18] Subsections 47(9) and (10) of the WSIA provide that if a worker with a permanent impairment greater than zero suffers a significant deterioration in his or her condition, and 12 months have passed since the Board's most recent determination concerning the degree of impairment, the worker may request that the Board redetermine the degree of the worker's permanent impairment. Board OPM Document No. 18-05-09, "Redeterminations" defines the term "significant deterioration" as follows:

A significant deterioration refers to a marked degree of deterioration in the work-related impairment that is demonstrated by a measureable change in objective clinical findings.

[19] The Board also supplied OPM Document No.18-05-09, "NEL Redeterminations."

(v) Testimony of the worker

[20] The worker testified to various problems with her knees. She testified that she wears a brace on her left knee as she has issues with "maltracking." She described how her knee "goes out." She testified that she has experienced multiple falls but she could not remember how many falls she has experienced.

[21] The worker testified that she has had various treatments for her knees including the use of braces, the application of hot and cold pads and anti-inflammatory creams. She has also had physiotherapy in the past but is no longer reimbursed for it by the WSIB.

[22] The worker testified that she has undergone surgery on both knees to repair meniscal tearing and patellar maltracking. She testified that she could have more invasive surgery on the left knee for the maltracking issue but is trying to delay this as long as possible and is having cortisone injections every three months as an alternative to further surgery.

[23] On cross-examination, the worker testified she had difficulty recalling the fall in February 2004. She recalled that she did fall while attending a program for the psychological trauma she experienced from the December 2002 assault and had a swollen left knee as a result. She recalled falling again in June 2004 on both her knees. She could not accurately recall when her left knee began to feel like it was being pulled to the side and was moving in the wrong direction as she described it.

[24] Upon questioning in examination in chief regarding various problems with her hands, she recalled problems with her left wrist, particularly after a fall in July 2013 when she recalled breaking the scaphoid bone in her left hand. She also recounted issues with bilateral Carpal Tunnel Syndrome (CTS) from approximately 2004 to 2006. She testified to now wearing a brace on her left wrist and hand. She received cortisone shots to her left hand. She testified that surgery has been suggested to remove various bones from her left wrist but she has not proceeded as doctors have advised that it might help her with pain but not with various functional problems. She testified that she believes her right hand is being overused as she must compensate for her problematic left hand when she undertakes bilateral hand actions.

[25] On cross-examination about her hands, the worker testified that she has overcompensated for left hand issues with her right hand, particularly in the last year or so.

[26] The worker testified that she must do all her household tasks very slowly because of issues with her hands. She cannot wash dishes. She is concerned that when she uses public transit, she will be jolted which will cause her knee pain.

[27] The worker testified to her life being negatively affected by all her health issues, especially in her knees and hands. She stated that she feels she has missed out on the best years of her life. She also testified that her relationship with her children has been adversely affected as she cannot do many activities with them. She also testified to various problems with her memory and concentration.

(vi) Submissions of the parties

[28] The worker's representative submitted that the condition of the left knee, caused by the slip and fall in February 2004, while the worker was attending treatment for the workplace injury of December 2002, was a significant contributing factor to secondary conditions in the right knee and in the bilateral hands, wrists and thumbs. This is based on the submission that the change in the worker's gait, caused by the compensable injury, led to instability in the left knee which was a significant contributing factor to overloading of the right knee which led to various right knee issues as secondary conditions.

[29] In addition, the worker's representative submitted that the instability in the left knee also caused the worker to experience numerous falls during which she injured her bilateral hands, wrists and thumbs such that conditions developed in these areas of her body which were secondary conditions causally related to the compensable left knee strain.

[30] The worker's representative submitted that the deterioration in the worker's left knee entitled her to a redetermination of the quantum of her 2006 NEL award for the left knee.

[31] The employer's representative submitted that the worker's conditions in her right knee and in her bilateral hands, wrists and thumbs were not secondary conditions to the compensable left knee strain. The employer's representative submitted that any instability in the left knee was not due to the compensable knee strain but was due to other non-compensable conditions and as such, the worker's right knee issues were not due to overcompensation for the left knee. The conditions in the worker's bilateral hands were also not due to the left knee compensable condition but to other non-compensable health issues.

[32] The employer's representative submitted that there had been no deterioration in the worker's Range of Motion (ROM) in her left knee since the 2006 NEL award. In addition, as she underwent surgery to her left knee in 2015, it was more likely than not that the left knee condition had improved and a NEL redetermination was not warranted.

[33] **(vii) Analysis**

[34] The appeal is denied for the reasons set out below.

[35] The applicable Board policy, that is, OPM Document No. 15-05-01, "Secondary Conditions Resulting From Work-Related Disability/Impairment" states that "... [entitlement] for any secondary condition is accepted when it is established that a causal link exists between it and the work-related injury."

(a) The left knee condition

[36] In her testimony at the hearing, I found that the worker was a poor historian. She could not remember the significant details of the numerous falls she had experienced. Nevertheless, she testified that the falls were all caused by the instability and "giving out" and "moving to the (other) side" of her left knee. The worker could not remember the specific mechanics of the compensable fall of February 4, 2004. Her left knee was initially damaged in that slip and fall for which the WSIB granted her entitlement to a left knee strain.

[37] An x-ray conducted on February 26, 2004, and read by Dr. E. Becker, radiologist, was compared to the first x-ray taken after the slip and fall of February 4, 2004. Dr. Becker reported as follows:

KNEE:

REFERENCE EXAMINATION: February 4, 2004.

There is a trace joint effusion I suspect. No fractures. **The alignment of the knee joint remains anatomic.**

There remains extensive soft tissue swelling and subcutaneous edema centered over the pre-patellar region, this spans over a distance of about 8 cm. Despite allowing for some technical variability, I suspect the swelling has increased since the previous study.

Again, I do not identify any fractures. The tibial plateaus appear intact. **The patella is unremarkable.**

IMPRESSION: Extensive prepatellar soft tissue swelling which has slightly increased since the previous study. This would be consistent with cellulitis or prepatellar bursitis.

(Bolding added by the Vice-Chair).

[38] I find that this x-ray revealed no damage to the alignment of the worker's left knee as a result of the compensable February 2004 fall. The worker's left knee was described as "anatomic." The patella was noted as "unremarkable." Therefore, I conclude the worker did not likely sustain any derangement or patellar damage in the compensable fall of February 4, 2004.

[39] Due to the worker's continuing experience of pain in the knee, she was seen by Dr. K. Wayne Marshall, orthopaedic surgeon, on February 26, 2004. Dr. Marshall wrote:

I reviewed [the worker] in the clinic today. It is still too difficult to examine her knee and she is complaining of severe pain and instability. We are therefore arranging for an MRI to determine whether or not she is having significant mechanical derangement.

[40] A MRI was conducted on the worker's left knee on March 2, 2004. Dr. Becker read this MRI as follows:

MRI LEFT KNEE

INDICATION: **Query cruciate ligament injury.**

REFERENCE EXAM: None.

FINDINGS: **The medial and lateral menisci appear intact and normal.** The cruciate ligaments and extensor mechanism appear normal.

There is a prepatellar bursa present which is thick-walled and contains some internal debris. It extends over the anterior and medial aspects of the patella and fluid is seen to track superiorly lying anterior to the distal quadriceps tendon.

There is an additional increased volume of fluid seen in the interval between the patellar tendon and the proximal tibia.

The patellar tendon itself has a normal signal, normal insertional appearance, and no focal marrow edema is seen.

There is very minimal increased signal within the medial collateral ligament consistent with a low-grade sprain injury.

Lateral collateral ligament appears normal.

The articular cartilage, bone marrow signal appear normal.

There is mild-to-moderate extraarticular soft tissue edema noted with patchy edema seen within both vastus medialis and vastus lateralis muscles.

IMPRESSION: Prominent prepatellar bursa which is thick-walled and contains some internal debris. **Intact cruciate ligaments. No meniscal tear.** Extraarticular soft tissue edema as described.

(Bolding added by the Vice-Chair).

[41] I find based on this x-ray and MRI evidence, that in the initial reporting and diagnosis of the left knee injury in February and March of 2004, the injury was consistent with a low-grade strain/sprain as determined by the diagnostic tests. There was no sign of left knee ligament damage or meniscal tearing which would affect stability and alignment of the joint. Accordingly, the WSIB granted entitlement for a left knee strain injury.

[42] The worker continued to be treated by Dr. Marshall, orthopaedic surgeon, for her left knee injury from the compensable accident of February 2004 and for her further non-compensable injury of the bilateral knees in another fall on June 25, 2004 which occurred when she tripped in a pothole.

[43] Dr. Marshall reported as follows about the worker on August 12, 2004:

I reviewed [the worker] in the clinic today. [She] is continuing to have pain in her knees which was exacerbated by a recent fall. However, there is still no indication for any surgical intervention and I have conveyed this to [her]. I do think that she is going to require long-term pain management.

[44] Dr. Marshall's reporting acknowledged that the worker perceived her pain as ongoing and severe in keeping with the conclusions of various treating specialists.

[45] Dr. Marshall further reported on November 11, 2004 that he was concluding his care for the worker which originated with the compensable left knee strain from the February 2004 fall:

I reviewed [the worker] in the clinic today. I have once again tried to emphasize with [her] that there is absolutely no indication for orthopedic surgery and indeed surgery would be highly likely to make her worse. I have discharged her from my care unless there should be further developments in which she is developing mechanical symptoms, which would lead us to consider repeat MRI.

[46] Despite the worker's claim that her left knee felt unstable after the February 2004 fall, Dr. Marshall was unable to examine her left knee stability due to her very high pain levels. Nevertheless, the contemporaneous diagnostic testing revealed no damage in the knee to which instability could be attributed.

[47] The employer's representative referenced the Tribunal Medical Discussion Paper (MDP), "Knee Conditions and Disability," August 2013, by Dr. John Cameron, Orthopaedic Surgeon, which contains the following paragraph on knee instability:

Instability

Many people complain of instability. It is important to determine whether the instability is secondary to a ligamentous injury, patella instability, or poor function of muscles around the knee due to pain.

[48] I find that the diagnostic testing indicated that the worker did not have a ligamentous injury from the compensable fall. I also find that she did not have patellar instability at that point, that is in November 2004, although there was some swelling in the prepatellar bursa. She was still suffering considerably from pain symptoms but when Dr. Marshall concluded his care of her left knee in November 2004, even after another non-compensable fall occurring in June 2004, he was clear that should she develop "mechanical symptoms," she could return to him for further care. From this statement, I infer that the worker was not then experiencing

mechanical symptoms in her left knee, such as instability. I place significant weight on the accuracy of the opinion of Dr. Marshall, since as a specialist in orthopaedic surgery, he is qualified to provide an opinion on diagnosis in this case; and since he saw and examined the worker prior to providing his opinion.

(b) Non-compensable falls and the worker's left knee after 2004

[49] I find that the evidence referenced another fall on June 25, 2004 which was non-compensable. The worker was not able to give detailed testimony about this fall. The WSIB Case Record revealed that this fall occurred when the worker tripped in a pothole and injured her left knee again, as well as spraining her left ankle. This fall was referenced above by Dr. Marshall in his reporting as he treated the worker for it also.

[50] The documents on file indicate that on March 1, 2006, a jolt of severe pain in the worker's back caused her to fall and break the fifth toe on her left foot. Further reporting from her family doctor, Dr. H. Obaji, on April 25, 2006, referred to the worker's fall in March 2006 after the worker "...had shooting pain from the left side going down to her leg. This caused her leg to go out...she fell fracturing her fifth toe." The worker's left knee is not mentioned as a causal factor in this report.

[51] On February 26, 2007, as outlined in WSIB Memorandum #125, the worker advised her WSIB Case Manager that she was hit by a car and had various injuries which aggravated her compensable conditions. There is little further detail about this incident in the Case Record.

[52] On September 18, 2008, the worker's longtime treating neurologist, Dr. O. Veidlinger, who began treating her for her migraine condition before the workplace accident in 2002, reported the following about the worker's back:

On examination today, there was tenderness over the neck. There was some decreased sensation over the lateral aspect of the left foot and dorsum of the foot and there was also slight weakness in dorsiflexion of the left foot and eversion of the left ankle, suggestive of an L5 root lesion... Back flexion was greatly limited. She has minimal signs of a left sciatica which should be managed conservatively.

[53] There is no mention in any of this later medical reporting about the worker's left knee being unstable or any causal relationship between the left knee and any of the worker's ongoing problems. At this point, I conclude that her right knee and her bilateral hands, wrists and thumbs were not symptomatic, based on the extensive contemporaneous medical reporting in 2008.

[54] I find there is no medical reporting of significance about the worker's left knee, continuing problems in the left knee or its effect on other parts of the worker's body between 2006 and 2013 when she fell and broke the scaphoid bone in her left hand. I find there is no objective medical reporting in the file of a significantly altered gait in the left knee or any reporting of adverse effects caused by altered gait in the left knee. I find that there is no medical evidence of significance relating problems of instability and altered gait in the left knee caused by the compensable injury.

(c) The condition of the worker's right knee

[55] The worker's representative submitted that patellar maltracking in the worker's left knee eventually caused an alteration in her gait which led to instability and the worker's multiple falls, as well as causing her to overload her right knee, triggering a secondary condition in it. I find that I do not accept this submission. I find that entitlement for the right knee as a secondary condition should be denied for the following reasons.

[56] First, I find that the medical evidence does not reveal a consistent pattern of instability and altered gait in the left knee over time such as to adversely affect the worker's right leg. The medical reports revealed a variety of instances where the worker made use of canes and crutches, dependent on the worker's acute symptoms at the time. I find that there is no medical evidence of significance to indicate that the worker settled into one pattern of left knee movement with an altered gait because of the compensable injury such as to adversely affect her right knee. Therefore, I am unable to find that altered gait in her left leg produced either instability such as to cause the various falls recounted or altered her gait to such a degree that her right knee was damaged.

[57] Second, I note that the Tribunal MDP, titled "Symptoms in the Opposite or Uninjured Leg," August 2005, by Dr. I. Harrington, orthopedic surgeon, contains the following statement about assumptions made that an injury to one leg will unduly stress the other leg. Dr. Harrington disputes this assumption except for those cases, unlike the worker's, where injury to one leg produces specific conditions over a long period of time which might lead to damage in the opposite leg:

Lay people and many doctors as well, believe that pain or disability in one leg can stress the other one and produce symptoms in it. It is often claimed that an injury causing disability of one leg initiated or aggravated a disabling condition in the opposite normal or previously asymptomatic lower extremity. It may be reasoned that the injury to one leg caused the patient to "favour" it and that this in turn unduly stressed the normal leg because it has had to bear more weight, causing or accelerating arthritis in one of its joints (usually the knee). It is assumed that when a person says he favours his leg, he means that he limps, sometimes requiring him to use crutches in order to protect the injured limb. The mechanics of limping are poorly documented in the orthopaedic literature and there is no clear scientific basis for such reasoning. In particular, few references have been found to the effect of the limp on the other leg. The evidence available indicates that an injury in one extremity rarely causes a major problem in the opposite or uninjured extremity except when damage to the leg results in a major displacement of the centre of gravity of the body while walking, significant shortening of the injured limb and the abnormal gait pattern has been present for an extended period of time.

(d) I accept this opinion as accurate

[58] In this appeal, I find there is no objective medical evidence of a prolonged alteration of the worker's gait, caused by the compensable left knee strain, such as would create overloading of the right leg to the degree necessary for the various issues which developed in the right knee after 2014 to be found to be a secondary condition caused by the compensable strain of the left knee.

(e) Treatment of the workers' knees after 2015

[59] I find that there is no medical evidence supporting a diagnosis of maltracking in the worker's left knee until after January 2015. Significant problems in the right knee were diagnosed in 2014 but were not treated surgically until 2016.

[60] There is further medical reporting regarding the worker's knees as they became more symptomatic in January 2015. Although a right knee MRI is referenced in Dr. Marshall's report below, it does not appear in the Case Record. On February 4, 2015, Dr. Marshall reported the following:

I reviewed [the worker] in the clinic today. The results of her right knee MRI disclosed a medial meniscal tear and evidence of patellar maltracking. She has patellar maltracking and fat pad impingement in her left knee. As an initial step, we are injecting each knee with 80 mg of DEPO-MEDROL and plan to review her again in 4 weeks' time.

[61] Dr. Marshall reported further on April 16, 2015:

[The worker] is continuing to struggle despite good conservative treatment. She has reached the point that she would like to proceed with arthroscopic surgery for bilateral patellar maltracking and torn right medial meniscus. I have made her aware of the risks including the risks of infection, DVT, PE, postsurgical bleeding and stiffness, neurologic injury, and failure of the procedure to alleviate symptoms.

[62] The worker underwent the left arthroscopic procedure on June 16, 2015. The following was set out in the post-operative report by Dr. Marshall:

PREOPERATIVE DIAGNOSIS:

1. Left lateral patellar maltracking.
2. Chondromalacia left patella.

POSTOPERATIVE DIAGNOSIS:

1. Left lateral patellar maltracking.
2. Chondromalacia patella.
3. Fat pad impingement left knee.
4. Fibrotic left medial plica.
5. Post-traumatic arthritis medial compartment left knee.

[63] On September 14, 2016, the worker underwent arthroscopic debridement of the right knee, arthroscopic partial right medial meniscectomy and arthroscopic partial lateral meniscectomy. The post-operative diagnosis was right patellar mal-tracking, fibrotic right medial plica, torn right medial meniscus, torn right lateral meniscus and osteoarthritis of the right knee.

[64] I find that there is no detailed medical reporting which links the left patellar maltracking to the right knee issues of meniscal tearing and patellar maltracking. The only medical reference to any causal connection between the left knee and the right knee is a brief statement given by Dr. Marshall in a report of December 3, 2015 where he stated the following:

I reviewed [the worker] in the clinic today. She is continuing to struggle and is increasingly having problems in her right knee due to the left knee overloading. She is going to continue to work on a strengthening program and we are renewing her physiotherapy. I plan to review her again in 2 months' time.

[65] I find that Dr. Marshall's brief reference to a possible connection between left knee overloading and the right knee problems does not address various problems of causation such as the passage of time between a minor strain injury to the left knee of 2004 and a significant diagnosis of multiple issues in the right knee many years later in 2016. In addition, Dr. Marshall does not address the evidence that indicates a likely contribution of various non-compensable injuries, such as the non-compensable fall of June 25, 2004, and age-related degenerative change to the condition of the worker's knees. I cannot conclude that the worker's right knee problems of 2016 are attributable to the fall of 2004 based on this brief statement by Dr. Marshall, given the lack of explanation provided.

[66] I also find that the worker did not testify in any detail to support that it was altered gait because of her left knee which caused problems in her right knee. Throughout her testimony, she was unable to recall the specifics of either her compensable fall of February 4, 2004 or her non-compensable falls to support that left knee instability and altered gait in the left leg were present consistently and had the causal effect of creating pain and damage in the right knee.

(f) The condition of the worker's bilateral hands, wrists and thumbs

[67] The worker is seeking entitlement to her bilateral hands, wrists and thumbs as secondary conditions to her left knee entitlement. The worker's representative submitted that the instability in the left knee caused various falls during which the worker broke her fall with her hands, causing numerous injuries to them.

[68] On July 13, 2013, the worker testified that she fell because of the instability in her knees and broke the scaphoid bone in her left hand. The worker attributed the problems in her hands, especially the left hand, to that fall of July 2013 and to numerous other falls, the dates of which the worker could not recall, which she testified were caused by claimed instability in her left knee.

[69] The worker testified that these non-compensable falls, particularly the one in July 2013, have seriously compromised her ability to use her hands, particularly her left hand. The worker could not recall any specific details of the July 2013 fall but testified that she has had to overuse her right hand to compensate for her reduced capacity to grip, hold and lift items with both hands as required in various tasks. The Case Record indicates in the worker's Form 6 that the worker is right-handed. Nevertheless, she testified that her right hand has experienced an overuse injury because of her increased reliance on it to compensate for her inability to complete tasks requiring bilateral hand use.

[70] On July 17, 2013, Dr. H. Alshahrani, radiologist, reported as follows regarding the worker's left hand:

[The worker] is referred from Emergency as a case of left scaphoid fracture. She has a history of a fall of her outstretched hand and came to Emergency 4 days ago and her x-ray showed an undisplaced fracture of the distal pole of the left scaphoid, so treated at that time by backslap. On examination she still has tenderness over the snuff box and neurovascular examination intact. The patient was seen and evaluated by Dr. Syed and the plan is to treat her with complete scaphoid cast for 6 weeks and see her after that to review the x-rays.

[71] On August 26, 2013, the worker was reassessed as part of the continuing treatment for her left scaphoid fracture. Dr. S. Srur, radiologist, reported as follows;

HISTORY AND EXAMINATION: Had followup of left scaphoid fracture 6 weeks ago. The patient is also complaining of the right wrist since the original injury, with mild tenderness over the snuff box. The cast was removed, showing no tenderness on the left side. **The x-ray of both wrists was normal.** So, the plan was reassurance and to be seen in the clinic on a p.r.n. basis. The case was seen and evaluated by Dr. Syed. (Bolding added by the Vice-Chair).

[72] In this medical reporting the right wrist is evaluated as being normal despite some complaints by the worker of pain in the right wrist. The testimony of the worker did not recount any of the mechanics of this fall to indicate that the right hand was involved; although, she did recount to doctors in 2013 and 2014 that she fell on her right hand as well as her left hand in the July 2013 non-compensable fall.

[73] Further medical reporting about the worker's hands in 2014 by Dr. A. Shararah, radiologist, reported as follows:

Date of Visit: 02 Oct 2014

[The worker] is a 50-year-old lady, right-hand dominant lady who presented with an injury to her left wrist for the suspicion of a left scaphoid fracture. She was casted for 6 weeks. She underwent MRI imaging of her left wrist to rule out ligamentous or bony injury to her wrist. **The MRI findings were negative for a scaphoid fracture or any ligamentous injury or tenosynovitis.** However, in keeping with her history, CMC arthritis of the first digit was evident. She is otherwise today complaining of pain on the radial aspect of her left hand, still in keeping with her osteoarthritis. That was to be managed conservatively for the time being with avoiding exacerbating activities and to continue with range of motion exercises as tolerated. (Bolding added by the Vice-Chair).

[74] This medical reporting related that the worker's left hand, wrist and thumb had evidence of osteoarthritis to which her pain and need for further treatment was attributed.

[75] I find that the Case Record contains no medical reporting of any significance regarding the condition of the worker's right hand as a whole or her right thumb or her right wrist. There is also little evidence of significance to indicate ongoing medical treatment for right hand conditions, other than the x-ray referenced above. Therefore, I am unable to conclude that the right hand, wrist or thumb have secondary medical conditions attributable to the left knee compensable injury.

[76] Despite her initial Emergency department diagnosis of left scaphoid fracture, I am unable to make a finding that this fall in July 2013 was caused by left knee instability attributable to the compensable accident for the reasons I have outlined above, including the fact that left knee instability is not a symptom related to the worker's compensable left knee injury. Therefore, I am unable to conclude that the left hand, left wrist or left thumb have secondary conditions attributable to the compensable left knee strain and entitlement for those conditions is denied.

(g) The medical opinions of the WSIB medical consultants

[77] Given the extensive medical issues and the extent of the worker's ongoing health issues in this appeal, the worker was the subject of several medical reports by WSIB Medical Consultants.

[78] I find these to be supportive of my conclusions that the worker does not have entitlement for injuries to her right knee and her bilateral hands, wrists and thumbs as secondary conditions to the left knee injury. The WSIB Medical Consultants' reporting surveyed all the various medical reporting from the worker's various specialists, most of whom treated her for different issues but were not aware of her ongoing symptoms and treatments as relayed to other treating specialists. The WSIB Medical consultants provided an overview of causality in their reports.

[79] Dr. J. Castiglione reported first on November 6, 2015:

My opinion is based on review of the file documentation without having had an opportunity to directly assess the worker, and may require revision should new information become available.

It is noted that this claim has accepted a left knee strain injury as a result of a fall while the worker was attending the PTP program in 2004 for her compensable workplace injury. I have been asked to provide my opinion on whether there would be instability within the accepted left knee impairment which would cause this worker to keep falling. If so, I am to provide my opinion on what diagnosis of the bilateral hand, wrist/ thumb would relate to the mechanism of injury. Alternatively, I am to comment on whether the ongoing bilateral hand, wrist/thumb injuries would be an outcome of a pre-existing

condition. Additionally, noting the April 16, 2015 Report from Dr. Marshall where he stated that the worker has patellar maltracking and a torn right medial meniscus, and given that the accepted entitlement is to a left knee strain, I am to provide my opinion on the relationship between the accident history, the 2004 left knee injury, and the current symptoms and diagnosis of the right knee.

The claim accepted a left knee strain injury related to a fall in 2004. There is little information on file with respect to the mechanism of the fall. There is no mention of any significant twist to support a meniscus tear, or any more significant ligamentous injury arising out of the fall to cause ongoing instability. In my opinion the accepted left knee impairment does not include instability and any ongoing instability in the worker's left knee cannot be objectively causally related to the 2004 fall or the initial workplace incident. As the instability cannot be causally related to the claim the bilateral hand/wrist/thumb injuries and OA cannot be causally related to the claim. The ongoing hand/wrist/thumb impairment appears to be most consistent with progression of OA in the worker's CMC joint.

As per Dr. Marshall's reports the worker has bilateral patellar maltracking and a right medial meniscal tear. MRI revealed an apical radial tear of the anterior horn of the right medial meniscus. There is a possible small fragment, anterior medially, associated with the anterior horn and there is focal moderate chondrosis of the central non-weightbearing portion of the medial femoral condyle. The worker has entitlement to a left knee strain. In a discussion paper prepared for WSIAT in 2005, Dr. Harrington noted that there is no clear evidence to suggest that an injury to one lower extremity would have any significant impact on the opposite uninjured limb unless the injury resulted in major muscle or nerve damage causing partial or complete paralysis of the damaged leg, and/or shortening of the injured lower extremity resulting in a limb length discrepancy of more than four or five centimeters so that the individual's gait pattern has been altered to the extent that clinically there is a significant limp which would need to be present over an extended period of time such as years. The compensable left knee strain did not result in a significant limp, leg length discrepancy, or nerve/muscle damage. In my opinion the right medial meniscal tear cannot be causally related to the compensable left knee strain or the original mechanism of injury. Patellar maltracking is typically due to muscle imbalances due to overuse or improper use. This worker's bilateral patellar maltracking is most consistent with an overuse syndrome or congenital muscle imbalance. In my opinion the patellar maltracking is not compatible with the reported mechanism of injury or sequelae in this claim. In my opinion the bilateral patellar maltracking, and right knee medial meniscus tear are not a claim responsibility.

[80]

In a further WSIB Medical Consultant's report, dated March 30, 2017, Dr. C. Gallimore addressed causality regarding the worker's asserted secondary conditions in her right knee and in her bilateral hands, wrists and thumbs:

1. In my opinion, the left knee patellar mal-tracking cannot be related or compatible with the work-related injuries. As noted by Dr. Castiglione, a patellar mal-tracking issue is more related to a muscle imbalance and cannot be attributed to a traumatic injury unless a direct impact had occurred on the patella which was not the case. Therefore, in my opinion, I am in agreement with Dr. Castiglione's original opinion.
2. Similarly, it is my opinion that the right knee overloading cannot be causally related to a left knee strain. As noted by Dr. Castiglione and in his review of the report, a significant impairment that would result in significant alteration in gait can cause injury in the opposite limb; however, this was not present in this case and therefore in my opinion the right knee cannot be directly attributed to the work injury.
3. The right knee surgery in my opinion cannot be attributed to the workplace injury or sequelae. The injury to the meniscus would require a rotational or compressive force and would not occur just from overloading.
4. In my opinion, the upper extremity injuries reported to the hand, wrist and thumb cannot be attributed to injury. There is no mechanism that would support these injuries.

5. In my opinion, with the left knee strain not significantly improving over time, the next step of care would have been the surgical procedure which occurred and therefore, in my opinion, the June 16, 2015 surgery would therefore be compatible. In terms of deterioration of the left knee, the NEL assessment reported a range of motion in knee flexion of 40°. I do not have any updated information that would suggest that there has been any deterioration in that range of motion. It is likely that the surgery likely would have improved and not worsened the condition and therefore, there is insufficient evidence to note that the worker's left knee has significantly deteriorated or to require a NEL redetermination at this stage.

(h) Conclusions regarding entitlement to secondary conditions

[81] I find that the worker has failed to establish on the balance of probabilities that the left knee strain which she experienced as a result of the compensable slip and fall of February 4, 2004 is the cause of the left patellar maltracking diagnosed eleven years after the compensable fall in January 2015. The medical reporting cited extensively above does not establish, on a balance of probabilities, a causal connection between the compensable left knee strain and the development of a change in gait of such a prolonged nature as to lead to a secondary condition in the worker's right knee. The worker has failed to establish how the right knee diagnoses, including right meniscal tears and right patellar maltracking, are related to the left knee condition. The evidence suggests it is more likely that the right knee diagnoses are based on non-compensable issues.

[82] I also find that the worker has failed to prove on the balance of probabilities that the bilateral hand conditions are secondary conditions causally connected to the compensable left knee condition. The worker has failed to prove that the instability in the left knee, which is alleged to have caused the worker to fall on her outstretched hands and injure them, is the responsibility of the 2004 compensable strain injury to the left knee.

[83] Therefore, the worker's appeal for entitlement for the right knee and the bilateral hands, wrists and thumbs as secondary conditions is denied.

(i) NEL redetermination for the left knee condition

[84] The worker also seeks a redetermination of the worker's 11% NEL award for her left knee due to a claimed deterioration of her left knee condition subsequent to the NEL award of August 23, 2006.

[85] Board OPM Document No. 18-05-09, "NEL Redeterminations" defines the term "significant deterioration" as follows:

A significant deterioration refers to a marked degree of deterioration in the work-related impairment that is demonstrated by a measureable change in objective clinical findings.

[86] The worker's representative submitted that the medical reporting regarding the worker's left knee after 2006 indicated a significant deterioration in her left knee condition such as to support a NEL redetermination. The employer's representative submitted that the treatment of the worker's left knee condition after her NEL award in 2006 was more likely than not to have improved her left knee condition overall. Therefore, the employer's representative submitted that, despite the worker's continued perception of worsening pain in the left knee, there was no objective medical evidence to support a finding of significant deterioration in the left knee such that a NEL redetermination was in order.

[87] I have little objective medical evidence regarding the worker's left knee condition since the 2015 surgery which was recognized by the WSIB as a claim responsibility. In the 2006 NEL Evaluation, the worker's ROM in the left knee in flexion was stated as 90 degrees. There is no updated reporting subsequent to the 2015 surgery on the ROM in her left knee.

[88] The only medical evidence which analyzed the possible impact of the 2015 left knee surgery on a possible NEL redetermination is the March 17, 2017 report of the WSIB Medical Consultant, Dr. Gallimore, who was queried about the impact of the surgery and reported as follows:

In my opinion, with the left knee strain not significantly improving over time, the next step of care would have been the surgical procedure which occurred and therefore, in my opinion, the June 16, 2015 surgery would therefore be compatible. In terms of deterioration of the left knee, the NEL assessment reported a range of motion in knee flexion of 40°. I do not have any updated information that would suggest that there has been any deterioration in that range of motion. It is likely that the surgery likely would have improved and not worsened the condition and therefore, there is insufficient evidence to note that the worker's left knee has significantly deteriorated to require a NEL redetermination at this stage.

[89] I note that it appears that Dr. Gallimore misstated the ROM of the knee in flexion as 40 degrees in the 2006 NEL assessment. The degree of flexion for the left knee in that report is 90 degrees. Nevertheless, I find that I am in agreement with Dr. Gallimore's conclusion that there is no objective medical evidence, since the 2015 left knee surgery, indicating that the condition of the left knee has significantly deteriorated such as to warrant a redetermination of the worker's NEL award for her left knee. As stated by Dr. Gallimore, the worker may even have experienced some improvement in the left knee as a result of the surgery.

[90] Should the worker suffer a significant deterioration in future, she may request a redetermination at the Board at that time, subject to the usual rights of appeal.

DISPOSITION

[91] The appeal is denied.

1. The worker does not have entitlement for secondary conditions in her right knee and bilateral hands, wrist and thumbs due to the compensable injury of her left knee.
2. The worker is not entitled to a redetermination of the degree of permanent impairment (quantum of the NEL award) for the left knee.

DATED: March 18, 2019

SIGNED: R.M.J. Hoare